

## PROTOCOL FOR HIV TREATMENT WITH ARVS

1. Treatment regimen adults, adolescents and pregnant women: 1st LINE ARV TREATMENT SCHEME (Use of 2 NRTI+1Integrase inhibitor)

| CATEGORIE DE POPULATION                                            | 1st year option recommended for all courses SANITARY | ALTERNATIVE TREATMENT FOR THE 1st LINE           |
|--------------------------------------------------------------------|------------------------------------------------------|--------------------------------------------------|
| Adults, adolescents and women of childbearing age or women speaker | TENOFOVIR/LAMIVUDINE/DOLUTEGRAVIR (TDF+3TC+DTG)      | ZIDOVUDINE/LAMIVUDINE+DOLUTEGRAVIR (AZT/3TC+DTG) |

Special precautions for pregnant women and women of childbearing age

Exclude pregnancy before prescribing a DTG-containing regimen and offer contraception to women of childbearing age

-In case of pregnancy, prescribe a regimen containing Dolutegravir and supplement the pregnant woman with **folic acid (1 Cp/dr)** for the duration of the pregnancy.

**If a pregnancy occurs in a woman on a DTG-containing regimen, the same regimen should be continued and the woman supplemented with folic acid.**

## 2. Recommended 1st-line regimen for INFANTS and CHILDREN

| Age groups                                             | 1st line option     | Alternative                                |
|--------------------------------------------------------|---------------------|--------------------------------------------|
| Newborn (under 4 Weeks)                                | AZT+3TC+RALTEGRAVIR | AZT/3TC+Nvp                                |
| 1month-6years(3-20Kg)                                  | ABC/3TC+DTG10mg     | AZT/3TC+DTG10mg                            |
| Over 6y-10y(20-30kg)                                   | ABC/3TC+DTG50mg     | ABC/3TC+ LPv/ritonavir or AZT/3TC+DTG 50mg |
| Over 10 years and Weight Greater than or equal to 30Kg | TDF/3TC/DTG         | AZT/3TC+DTG 50mg                           |

### **3 Prophylactic treatment**

A primary prophylaxis against opportunistic infections

In children born exposed; COTRIMOXAZOLE prophylaxis is recommended from 6 weeks to 18 months if serology becomes negative at 18 months.

In adults, COTRIMOXAZOLE prophylaxis should be initiated as soon as HIV serology is positive,

#### **B Preventing tuberculosis in PLHIV**

-Screen all newly diagnosed PLHIV for TB in order to eliminate active TB

-Start INH prophylaxis if screening is NEGATIVE( 10-15 mg/Kg/dr for 6 months)

-If the screening is POSITIVE, active TB must be investigated (BK research and GeneXpert test).